INFORMATION TO KEEP WITH YOU

My complete treatment handbook

FOLFIRINOX followed by chemoradiotherapy · rectal cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00691
Version 4

Phase 1: six FOLFIRINOX cycles, one every 14 days, each with a 46-hour fluorouracil pump. Phase 2: capecitabine twice daily on Days 1–5 of each seven-day week for five weeks, with radiotherapy. Your team supplies the separate radiation calendar.

Treatment stages

01 **Phase 1 · six FOLFIRINOX cycles**
Day ward → home pump → review

02 **Phase 2 · five weeks with radiotherapy**
Capecitabine tablets + pelvic radiotherapy

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever, shivers or sudden illness needs immediate oncology advice. Chest pain, collapse or severe breathing difficulty needs emergency help. Call urgently for severe or persistent diarrhoea, severe abdominal pain, inability to drink, reduced urine or a line/pump problem.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

This is a planned course before surgery assessment for locally advanced rectal cancer. Six FOLFIRINOX cycles are followed by capecitabine with pelvic radiotherapy. The team reviews response and the next part of your overall rectal-cancer plan.

PLATINUM CHEMOTHERAPY

Oxaliplatin

Eloxatin



Links deoxyribonucleic acid (the cell's genetic material) (DNA) strands

Oxaliplatin makes links within DNA. These interfere with copying and repair, which can stop a cancer cell dividing. Healthy tissues can also be affected.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Cold can trigger tingling or throat discomfort. Tell the team about persistent numbness, difficulty with buttons or walking, or infusion symptoms. New breathing difficulty always needs assessment.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

TOPOISOMERASE I CHEMOTHERAPY

Irinotecan

Campto



Interrupts DNA copying

The body converts irinotecan into SN-38. This traps an enzyme called topoisomerase I while DNA is being copied, causing damage that can stop cancer cells dividing. Normal bowel and blood-forming cells can also be affected.

Irinotecan becomes SN-38 → DNA-repair enzyme is trapped → Dividing cells are damaged

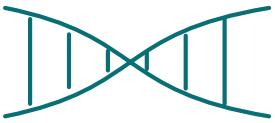
What to watch for

Tell the nurse immediately about cramps, sweating or diarrhoea during the infusion. Diarrhoea starting later needs the separate written treatment and contact plan.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with the production and use of DNA and RNA, the instructions cells need to grow. The infusion extends exposure over approximately two days.

Drug enters cell → DNA / RNA disrupted → Growth slows

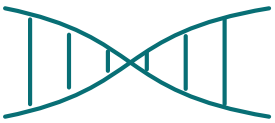
What to watch for

Diarrhoea, mouth sores and skin soreness may become serious. Chest pain during the pump infusion needs emergency assessment. Unexpected severe early toxicity can occur even after dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing.

ORAL CHEMOTHERAPY

Capecitabine

Xeloda



Becomes fluorouracil in the body

After you swallow capecitabine, your body changes it into fluorouracil. This interferes with the DNA and RNA processes cells use to grow. Normal cells can also be affected, which helps explain mouth soreness, bowel symptoms and changes in blood counts.

Swallowed tablet → Converted to fluorouracil → Cell growth disrupted

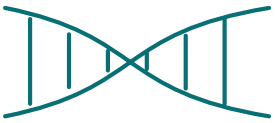
What to watch for

Stop tablets and contact the team for significant diarrhoea, painful mouth ulcers, hand-foot soreness affecting activities, fever or dehydration. Chest pain needs urgent assessment. Do not restart until instructed.

LOCAL RADIATION TREATMENT

Pelvic radiotherapy

External beam treatment



Targets the pelvic treatment area

Radiation is directed at the planned area. Capecitabine helps sensitise cancer cells to it. Nearby bowel, bladder and skin may also become irritated. Effects can build over the weeks and may continue after the final session.

Planned radiation field → Damage to cancer-cell DNA → Cells lose the ability to multiply

What to watch for

Tell the radiotherapy team about diarrhoea, pain, skin soreness or urinary changes. Discuss possible longer-term bowel, bladder, sexual and fertility effects before treatment.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and evidence

The setting and sequence matter.

What studies can tell us

PRODIGE 23 studied the whole treatment strategy: six FOLFIRINOX cycles, chemoradiotherapy, surgery and postoperative chemotherapy. It improved disease-free survival compared with the comparator strategy of chemoradiotherapy, surgery and postoperative chemotherapy without initial FOLFIRINOX. The result cannot be attributed to induction chemotherapy alone and is not a personal prediction of cure.

Review and your priorities

Ask when benefit will be reviewed and which symptoms, scans or tests will be used. Discuss work, caring responsibilities, time at home and what matters most to you. Delaying or reducing a medicine can be necessary for safety and does not itself prove treatment has failed.

Understanding the evidence

Ask which study population and treatment sequence match your plan.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment plan

Treatment stages

01 **Phase 1 · six FOLFIRINOX cycles**
Day ward → home pump → review

02 **Phase 2 · five weeks with radiotherapy**
Capecitabine tablets + pelvic radiotherapy

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAY WARD → HOME PUMP → REVIEW

Phase 1 · six FOLFIRINOX cycles

C1 C2 C3 C4 C5 C6

FOLFIRINOX every 14 days for six cycles; each includes a 46-hour fluorouracil pump.

CAPECITABINE TABLETS + PELVIC RADIOTHERAPY

Phase 2 · five weeks with radiotherapy

W1 W2 W3 W4 W5

Capecitabine twice daily on Days 1–5 of each seven-day week for five weeks, with radiotherapy. The radiation unit provides the appointment calendar; the team confirms any treatment-day changes.

Six induction cycles are followed by a separate five-week chemoradiotherapy phase. The multidisciplinary team confirms restaging, surgery assessment and any later treatment; these are not additional FOLFIRINOX cycles.

BASELINE → TREATMENT → FOLLOW-UP

Required and conditional checks across your treatment

Required checks are shown explicitly. electrocardiogram (ECG), additional urine tests and anticoagulant tests apply only when indicated. Scans and tumour markers follow your individual cancer plan; this page does not prescribe a universal surveillance interval. Confirm who books and reviews each test.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00691 v4 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

FOLFIRINOX followed by chemoradiotherapy · rectal cancer: treatment and checks

Locally advanced rectal cancer

Phase 1: six FOLFIRINOX cycles, one every 14 days, each with a 46-hour fluorouracil pump. Phase 2: capecitabine twice daily on Days 1–5 of each seven-day week for five weeks, with radiotherapy. Your team supplies the separate radiation calendar.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Review / change	After treatment
Blood counts / kidneys / liver NCCP 00691	Required baseline blood count, kidney and liver tests	Before every cycle: each 14-day FOLFIRINOX cycle and each weekly capecitabine cycle	Earlier tests if unwell, dehydrated or toxicity develops	Recovery and results guide delay or dose adjustment	Follow unresolved abnormalities as agreed
DPD / fluoropyrimidine safety NCCP 00691	DPD test before fluorouracil or capecitabine unless already tested	The result guides the starting plan; it does not exclude all severe toxicity	Unexpected severe early mouth sores, diarrhoea or illness needs urgent assessment	The team reviews suspected severe reactions before further treatment	Keep the result and any toxicity history in your records
Nerves / cold sensitivity NCCP 00691	Record any pre-existing numbness or difficulty with daily tasks	Neuropathy assessment every cycle for this regimen	Report persistent tingling, trouble with buttons or walking; avoid cold triggers	Oxaliplatin may be changed separately after clinical review	Nerve symptoms can persist; arrange follow-up if unresolved

Care / check	Before starting	During treatment	Between visits	Review / change	After treatment
Heart assessment NCCP 00691	Review cardiac history; baseline ECG if cardiac function is compromised	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
Radiotherapy / tablets NCCP 00691	Confirm planning appointments and the phase-2 prescription	Review bowel, bladder, skin, hydration and tablet tolerance during chemoradiotherapy	Report worsening pelvic symptoms; follow the local radiation preparation plan	Do not restart held tablets or make up missed doses without instructions	Multidisciplinary restaging and surgery assessment are individual; follow late pelvic effects
Response / scans / tumour markers NCCP 00691	Agree a baseline disease assessment and treatment aim	Review symptoms, function and clinical benefit	New symptoms may require earlier review	Scans and tumour markers follow the consultant-directed cancer plan	Agree next treatment or follow-up; there is no universal scan interval in this regimen

What each check is for

1 Phase 1 · six FOLFIRINOX cycles

Day ward → home pump → review

This is a planned course before surgery assessment for locally advanced rectal cancer. Six FOLFIRINOX cycles are followed by capecitabine with pelvic radiotherapy. The team reviews response and the next part of your overall rectal-cancer plan.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

2 Phase 2 · five weeks with radiotherapy

Capecitabine tablets + pelvic radiotherapy

This is a planned course before surgery assessment for locally advanced rectal cancer. Six FOLFIRINOX cycles are followed by capecitabine with pelvic radiotherapy. The team reviews response and the next part of your overall rectal-cancer plan.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Choose and prepare the line

The team explains whether a port or peripherally inserted central catheter (PICC) suits your treatment. You receive instructions for the insertion appointment.



2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.



3 · Day 1

The nurse connects your pump

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.



4 · At home

Treatment continues for about 46 hours

Carry the pump as shown. Keep the dressing dry and protect the line from pulling, heat and damage.



5 · As arranged

Return for disconnection

A trained professional checks the infusion, disconnects the pump and cares for the line. The appointment is based on connection time.



6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Confirm the aim, DPD result, test appointments, prescribed supportive medicines and working urgent contact numbers. Bring your full medicines list, including supplements.

STEP 2

Line, pump and disconnection

Agree the access device and care training. The pump runs for 46 hours; confirm the disconnection appointment from its connection time. Do not alter settings, disconnect or reconnect it unless specifically trained and instructed.

STEP 3

If the plan changes

The team confirms the next treatment, blood-test and supportive-medicine dates after a delay or dose change. Do not use a previous calendar as a prescription.

STEP 4

Contacts, paper copies and QR

The example telephone numbers on the wallet card are placeholders. Your team must replace them with working daytime and out-of-hours numbers before patient use. Keep the card with you; the QR opens the regimen information, not your confidential medical record.

Your treatment schedule

Use your prescription and team-confirmed calendar.

Medicines and sequence

Phase 1 is six cycles every 14 days: oxaliplatin 85 mg/m² and folinic acid 400 mg/m² (200 mg/m² may be considered), irinotecan 180 mg/m², then fluorouracil 2,400 mg/m² continuously over 46 hours; no fluorouracil bolus. Phase 2 is capecitabine 800 mg/m² twice daily on Days 1–5 of each seven-day cycle for five weeks with radiotherapy. NCCP permits a consultant-selected 825 mg/m² alternative; your actual tablet prescription and radiation calendar govern treatment.

How long treatment lasts

Six FOLFIRINOX cycles are followed by five weeks of capecitabine with radiotherapy. The multidisciplinary team then confirms assessment and next steps.

The disconnection appointment

The fluorouracil infusion lasts 46 hours from connection. Arrange the completion visit before leaving the unit. Pump removal does not remove an implanted port: the port remains until a separate procedure is arranged.

Before, during and after treatment

Testing detects problems that may not cause symptoms.

Before the first dose

Blood count, kidney and liver tests and DPD testing are required before fluoropyrimidine treatment unless the DPD test has already been done. DPD helps the body clear fluorouracil; deficiency changes the treatment plan. A normal result does not exclude all severe toxicity.

Before each cycle

Blood count, kidney and liver results and symptoms are reviewed before each cycle. In this two-phase plan that means each 14-day FOLFIRINOX cycle and each weekly capecitabine cycle. Neuropathy is assessed every cycle; report troublesome symptoms sooner. Missing results are not assumed normal.

Heart tests are conditional

Tell the team about heart disease and previous chest symptoms. An ECG is needed when cardiac function is compromised or clinical findings warrant it. There is no automatic recurring echocardiogram schedule for every patient.

Scans, markers and aftercare

Your consultant decides response scans, tumour-marker use and follow-up according to your cancer and treatment aim. This regimen does not set one scan or marker interval for everyone. Unresolved side effects or abnormal blood results need a follow-up plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day-ward medicines · induction	 **Day 1** intravenous (IV)
Fluorouracil pump	 **Days 1–3** 46 hours; disconnect as arranged
Early cramps / sweating	 **Day 1** Tell the nurse immediately
Delayed diarrhoea	 **Days 2–14** May start after 24 hours; follow written plan
Cold sensitivity / nerves	 **Days 1–14** Can occur early and accumulate; report persistent symptoms
Blood counts / infection	 **Days 1–14** No guaranteed safe days
Mouth / bowel / energy	 **Days 1–14** Timing varies; record and report symptoms

Day ward and home pump: Day 1 infusions lead into a 46-hour pump. The disconnection visit is arranged from the actual connection time.

No safe window: Fever, sudden illness or a serious symptom needs action on any day, including after pump removal.

Phase changes: Six induction cycles are followed by a separate five-week chemoradiotherapy phase. The multidisciplinary team confirms restaging, surgery assessment and any later treatment; these are not additional FOLFIRINOX cycles.

Two different bowel reactions: Cramps and sweating during infusion need immediate nursing attention. Diarrhoea starting more than 24 hours later needs the separate written home plan and early contact.

Between visits: Record symptoms and their effects on drinking, eating, bowel/stoma output, mobility and usual activities.

> **Why these bands stay broad:** This graph shows a 14-day FOLFIRINOX induction cycle. The subsequent capecitabine/radiotherapy phase follows a separate seven-day weekly schedule shown in the phase plan; do not use the induction graph to decide tablet days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00691 v4 · official regimen ↗

> **There are no guaranteed "safe days".** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1: Infusion and pump connection

The team checks symptoms, blood results and the active treatment plan. Day-ward infusions are followed by connection of the fluorouracil pump. Before leaving, confirm the pump-care instructions, disconnection appointment and contact number.

Day 2–3: The pump is delivering treatment

The pump runs for approximately 46 hours from connection. Keep the device and line secure and dry as taught. The team confirms the actual disconnection appointment. Call for a leaking, disconnected or unexpectedly stopped pump. Do not reconnect, alter settings or disconnect it yourself unless specifically trained and instructed.

Day 4–7: After disconnection

Delayed diarrhoea can begin after the first 24 hours and may continue after pump removal. Use your written plan and contact the team early. Cold-triggered tingling may occur and nerve symptoms can persist. Nausea and tiredness can overlap; symptoms are not restricted to these days. Report poor intake, persistent diarrhoea or mouth pain early. Chest pain or major breathing difficulty requires emergency help.

Day 8–14: Between visits and the next review

Blood counts can fall between visits. Tell the team about bowel symptoms, mouth pain and how you are managing daily activities. Treatment proceeds only after review. Confirm the next induction cycle only after review. After cycle 6, ask for the separate capecitabine and radiotherapy calendar; do not continue FOLFIRINOX automatically.

Support around the medicines

1 Nausea prevention and rescue

This chemotherapy combination has a high risk of sickness without prevention. Follow the prescribed prevention and rescue medicines. Call if vomiting prevents drinking; do not duplicate steroids or anti-sickness doses.

2 Blood counts and recovery

Growth-factor injections depend on your risk, blood counts and previous complications. If prescribed, confirm their timing in relation to chemotherapy and pump removal; do not assume everyone receives them.

3 Early and delayed diarrhoea plans

Atropine is normally prescribed before irinotecan for early cramps, sweating or diarrhoea. It is avoided with glaucoma; tell the team about asthma as well. At home use the separately prescribed delayed-diarrhoea instructions. This can differ from standard over-the-counter dosing. Keep the medicine and urgent-contact plan available.

4 Cold and mouth care

Avoid cold drinks, cold air and cold objects if they trigger symptoms. Do not use ice chips during oxaliplatin treatment. Ask for non-cold mouth-care advice. New breathing difficulty must still be assessed rather than assumed to be a cold reaction.

5 Support during chemoradiotherapy

Capecitabine alone has a lower sickness risk than the induction combination, so the supportive prescription changes with the phase. Ask about pelvic skin care, bowel and bladder symptoms, diet and radiotherapy preparation.

Bowel symptoms and hydration

Act early; do not wait for the next appointment.

Keep the written plan

Record the change from your usual bowel pattern, including stoma output. Keep the prescribed diarrhoea medicine and your oncology contact details available. Ask for specific instructions before the first treatment.

Two different irinotecan reactions

During or soon after infusion, cramps, sweating, a runny nose or diarrhoea can be an early cholinergic reaction: tell the nurse immediately. Diarrhoea starting more than 24 hours later needs the separate prescribed delayed-diarrhoea plan, which may differ from packet instructions. If it persists despite that plan, or you become unwell, seek urgent advice.

Urgent symptoms

Contact oncology urgently for persistent diarrhoea, inability to drink, reduced urine, dizziness, blood in stools, fever or severe abdominal pain. Do not assume severe abdominal pain is an ordinary treatment effect. A stoma can make fluid loss less obvious: report a marked increase in output.

Cold sensitivity and longer-lasting nerve symptoms

These are two related but different oxaliplatin problems.

Cold-triggered symptoms

Cold drinks, cold air and cold objects can bring on tingling or throat sensations. Avoid triggers and do not use ice chips during oxaliplatin treatment. New breathing difficulty still needs urgent assessment; it must not be diagnosed at home as simply a cold reaction.

Persistent neuropathy

Report numbness, tingling, pain or difficulty doing buttons, writing, holding objects or walking. Symptoms can accumulate over cycles and sometimes persist after treatment. Explain how long symptoms lasted and what activities changed.



Review before more oxaliplatin

Nerve assessment is required each cycle for this regimen. The team may change or stop oxaliplatin independently of the other medicines to reduce the risk of lasting harm. Do not wait for a scheduled assessment if symptoms are worsening.

The second phase: tablets and pelvic radiotherapy

This has its own calendar and side-effect plan.



Tablet days and dose

After six FOLFIRINOX cycles, NCCP describes capecitabine 800 mg/m² twice daily on Days 1–5 of each seven-day cycle, for five weeks with radiotherapy. A consultant may select the specified 825 mg/m² alternative. The pharmacy label states your tablet strengths and number of tablets; do not calculate your own dose from this handbook.

Taking capecitabine

Swallow tablets whole with water within 30 minutes after food. Do not cut, crush or chew them. If a dose is missed or vomited, do not take a replacement or double the next dose; record it and follow the team's advice.

If treatment is held

Stop capecitabine and contact the team for significant diarrhoea, painful mouth ulcers, hand-foot soreness affecting activity, fever or dehydration. Chest pain needs urgent assessment. Do not restart until instructed. If radiotherapy is cancelled or changed, ask whether that day's tablets change too.

Pelvic effects and preparation

Radiotherapy may affect the bowel, bladder and skin in the treatment area, and can have later effects on bowel function, sexual function and fertility. Ask for the radiation unit's bladder/bowel preparation, skin-care and symptom plan. Report urinary pain, worsening pelvic symptoms or skin breakdown.

After the two phases

The multidisciplinary team arranges assessment of response and suitability for surgery or other next steps. The cited trial included surgery and postoperative chemotherapy, but these are separately agreed parts of your overall plan rather than automatic extra cycles in this document.

Mouth, skin, energy and infection

Frequency and urgency are different.

Mouth and hands or feet

Fluorouracil and capecitabine can cause mouth ulcers and painful redness or peeling of hands and feet. Report symptoms early, especially if eating, drinking, walking or using your hands becomes difficult. Ask for non-irritating mouth care and skin-care advice.

Nausea, appetite and fatigue

Use the anti-sickness prescription and call if it is not controlling symptoms. Small regular meals and manageable activity with rest may help daily routines. Fatigue can have several causes; persistent or worsening fatigue may need blood tests or review. Irinotecan may also cause hair thinning or loss; ask about practical support.

Low blood counts and infection

Chemotherapy can reduce white cells, red cells and platelets. Fever, shivers or sudden illness needs immediate oncology advice, even if your temperature is not raised. Follow the unit's temperature instructions and do not wait for a scheduled blood test. Unusual bruising, bleeding, breathlessness or marked weakness also needs review.

Heart and infusion symptoms

Fluorouracil and capecitabine can rarely cause serious heart problems. Chest pain, collapse or severe breathing difficulty needs emergency assessment. Tell the nurse immediately if flushing, swelling, wheeze, rash or pain occurs during an infusion.

Your line and home pump

Ask for a demonstration and show the team what you would do.

Access and ordinary care

The team selects a port, PICC or other suitable access with you. Keep dressings and connections secure and follow the specific washing, bathing and activity instructions. Keep the pump away from excessive heat or cold.

Leak, disconnection or unexpected flow

Call the oncology contact for a leaking or disconnected pump, pain/swelling at the line, a pump that finishes early, or one that appears not to be delivering. Do not change settings or reconnect or disconnect the device unless specifically trained and instructed. Follow the spill/emergency steps the unit has taught you.

Disconnection and future care

The pump is disconnected at the arranged visit and the line is cared for. A port needle can be removed while the implanted port stays under the skin. Ask when any dressing changes, flushes or later line removal are due.

Supportive medicines and daily life

The prescription changes with the active treatment.

Your supportive prescription

Confirm prevention and rescue medicines for nausea, the diarrhoea plan and any growth-factor injections. Atropine may be used before irinotecan for the early reaction; mention glaucoma and asthma. Do not duplicate anti-sickness medicines, start supplements or take leftover prescriptions without checking with the oncology pharmacist.

Medicines and interactions

Bring all prescribed and non-prescribed medicines, supplements and anticoagulants to review. Tell the team before starting antibiotics or a new medicine. Do not independently stop regular treatment such as anticoagulants before a procedure.

Fertility, pregnancy and sex

Discuss fertility preservation before starting where relevant. Ask which contraception, pregnancy, breastfeeding and sex precautions apply during treatment and after the last dose. The period depends on your medicines and circumstances.

Practical support

Ask about work, driving, travel, nutrition, caring responsibilities and emotional support. Report symptoms in terms of what you can eat, drink or do. An interpreter, another explanation or a written plan can help you and a chosen family member or carer.

Your paper plan, wallet card and contacts

The three-page summary and handbook are usable without a phone.

Cycle and phase record

Ask the team to stamp or write the treatment actually given. The six card spaces apply only to FOLFIRINOX induction. Record the following five chemoradiotherapy weeks separately in the passport; do not count them as more FOLFIRINOX cycles.

Before leaving

Confirm your next tests and appointments, pump disconnection when relevant, medicines to take at home and how to obtain urgent help. Explain the plan back in your own words; ask for anything unclear to be written down.

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Will I need a port or a PICC?

The team chooses the most suitable access device with you. A port is under the skin; a PICC enters through the upper arm and has an external section. Both require specific care, and the pump can be used with either.

Does disconnecting the pump remove my port?

No. At the arranged visit the pump is disconnected and the line cared for. A port needle may be removed, but the implanted port stays under the skin until a separate removal procedure is arranged.

What if the pump finishes early or still looks full?

Contact the team for advice. Do not change delivery settings or disconnect or reconnect it yourself. Follow only the specific emergency steps you have been taught.

Is delayed diarrhoea the same as cramps during infusion?

No. Irinotecan can cause an early reaction with cramps, sweating or diarrhoea, and a later diarrhoea problem. Tell the nurse about early symptoms and follow the separate written plan at home.

Does the wallet QR contain my treatment record?

No. The printed QR opens this regimen's patient information. Your cycle stamps, appointments, test results and treatment changes remain in your own records and the clinical record.

Can I use the printed information without a phone?

Yes. The three-page summary, full handbook and passport can be printed. Ask your team to write the next appointments and working contact numbers on the paper copies.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

FOLFIRINOX induction: six 14-day cycles

Active medicines: Oxaliplatin + Folinic acid + Irinotecan + Fluorouracil

Emetogenic risk: High

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 before chemotherapy Check interactions; national dexamethasone doses below already account for this combination.

Medicine	Dose / route	When
Aprepitant	80 mg orally once daily	Days 2 and 3 Do not duplicate another NK1 antagonist.
Ondansetron	16 mg orally once	Day 1 before chemotherapy Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	12 mg orally once	Day 1 before chemotherapy Reconcile all steroids; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2, 3 and 4 Re-anchor to the actual treatment day if delayed.
Olanzapine	5 mg orally once daily	Days 1, 2, 3 and 4 Review sedation, falls, glucose and interactions.
Metoclopramide	10 mg orally up to three times daily as required	Subsequent-day rescue; prescriber confirms duration Review renal function and neurological contraindications. Concomitant olanzapine increases extrapyramidal adverse-effect risk; reconcile rescue medicines.
Atropine sulphate	250 micrograms subcutaneously	Before irinotecan if prescribed Avoid in glaucoma; use caution with asthma. This treats acute cholinergic effects, not delayed diarrhoea.

Additional phase-specific support

- This chemotherapy combination has a high risk of sickness without prevention. Follow the prescribed prevention and rescue medicines. Call if vomiting prevents drinking; do not duplicate steroids or anti-sickness doses.
- Growth-factor injections depend on your risk, blood counts and previous complications. If prescribed, confirm their timing in relation to chemotherapy and pump removal; do not assume everyone receives them.
- Atropine is normally prescribed before irinotecan for early cramps, sweating or diarrhoea. It is avoided with glaucoma; tell the team about asthma as well. At home use the separately prescribed delayed-diarrhoea instructions. This can differ from standard over-the-counter dosing. Keep the medicine and urgent-contact plan available.
- Avoid cold drinks, cold air and cold objects if they trigger symptoms. Do not use ice chips during oxaliplatin treatment. Ask for non-cold mouth-care advice. New breathing difficulty must still be assessed rather than assumed to be a cold reaction.
- Capecitabine alone has a lower sickness risk than the induction combination, so the supportive prescription changes with the phase. Ask about pelvic skin care, bowel and bladder symptoms, diet and radiotherapy preparation.
- Irinotecan delayed-diarrhoea rescue must be supplied as a written prescription: loperamide 4 mg initially, then 2 mg every 2 hours until 12 hours after the last liquid stool, for no longer than 48 hours at this high-dose schedule. Fever, dehydration, severe pain or persistent diarrhoea needs urgent oncology assessment. Do not carry this schedule into capecitabine treatment without a new prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Capecitabine with radiotherapy: separate five-week phase

Active medicines: Capecitabine + Radiotherapy

Emetogenic risk: Oral minimal-to-low; radiation risk assessed separately

Use oral-systemic anticancer therapy (SACT) antiemetics as required under local policy. The radiation team assesses radiation-related emesis separately. The induction aprepitant, dexamethasone, olanzapine and atropine schedule does not continue automatically into this phase.

Additional phase-specific support

- Confirm the capecitabine prescription and radiation-day calendar: planned Days 1–5 each week for five weeks; no catch-up doses without instructions.
- Capecitabine is taken within 30 minutes after food. Use the written missed-dose, vomiting and diarrhoea plan.
- Confirm weekly blood tests and bowel, skin, hydration and radiation review. Stop capecitabine and contact the team for significant toxicity as instructed.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth / swallowing

Fluorouracil + folinic acid / later capecitabine

Report ulcers or pain that affects eating or drinking.

2 Chest

Fluorouracil / capecitabine

New chest pain, collapse or major breathing difficulty needs emergency help.

3 Bowel / stoma

Irinotecan / fluorouracil / radiotherapy

Call early for diarrhoea, increased stoma output or poor intake; severe pain needs urgent assessment.

4 Nerves / cold

Oxaliplatin

Avoid cold triggers; report persistent numbness or difficulty with buttons or walking.

5 Blood / infection

Chemotherapy

Fever, shivers or sudden illness: call now. Unusual bruising or marked weakness also needs advice.

6 Hands / feet

Capecitabine / fluorouracil

Painful redness or peeling affecting activity: hold capecitabine and call during the tablet phase.

7 Pelvis / bladder

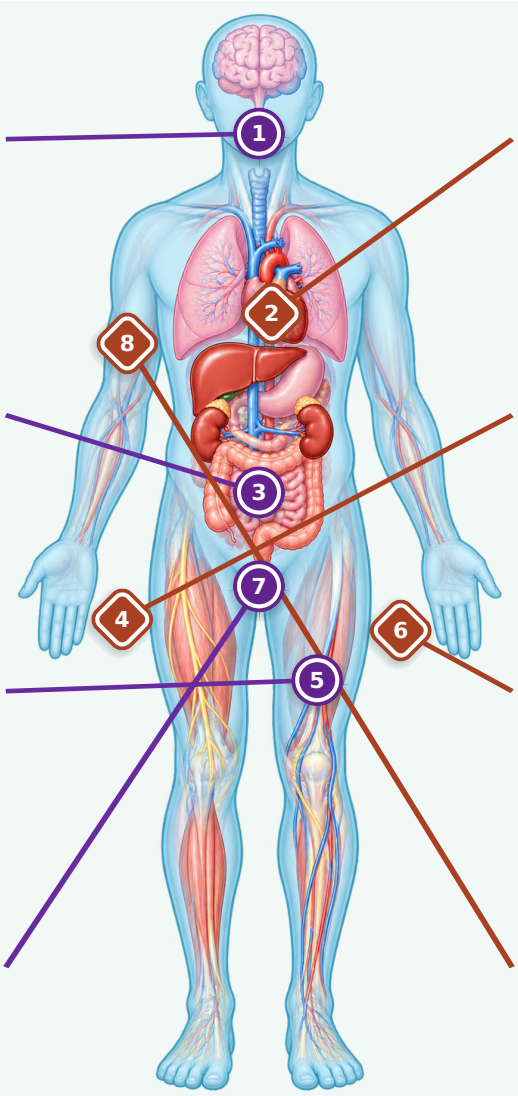
Pelvic radiotherapy

Report painful urination, worsening pelvic symptoms or sore treatment-area skin.

8 Pump / line

Treatment delivery

Call for leakage, swelling, line pain or a loose connection. Do not reconnect it yourself.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or sudden illness needs immediate oncology advice. Chest pain, collapse or severe breathing difficulty needs emergency help. Call urgently for severe or persistent diarrhoea, severe abdominal pain, inability to drink, reduced urine or a line/pump problem.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00691 v4 · official regimen**
   https://healthservice.hse.ie/documents/6531/691_v4_Folfirinox_Rectal_carcinoma.pdf
2. **eviQ: implanted port**
   https://www.eviq.org.au/p/507
3. **eviQ: peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
4. **eviQ: infusion pump care**
   https://www.eviq.org.au/p/502
5. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **UNICANCER-PRODIGE 23: Conroy et al. 2021**
   https://pubmed.ncbi.nlm.nih.gov/33862000/
7. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
8. **NCCP NCIS antiemetics V8: high-risk non-AC row; multiday support reviewed by prescriber**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf